

Scenario 1: Opioid

Background

Weather: Sunny, mid-spring evening, 70°F with low humidity.

Dispatch

Reported Incident: An unconscious person was found in a gas station bathroom.

Bystander Report: The patient was found unresponsive with an empty pill bottle.

On Scene

Patient: Young adult male in his 20s.

Condition: Shallow respirations, bradycardic pulse, pinpoint pupils.

History of Present Incident

Bathroom Access: The gas station attendant gave the bathroom key to the patient approximately 15 minutes before discovering him unconscious.

Primary Survey

Airway and Breathing: Patient is unconscious with shallow respirations.

Circulation: Weak pulse.

Mental Status: Unconscious.

Other Observations: Empty pill bottle found nearby, indicating possible opioid overdose.

Initial Physical Exam

Pulse: 55 bpm

Respiration: 12/min

Blood Pressure: 120/70 mmHg

SPO2: 94%

Glucose: 109

Treatment & Transport

Immediate Treatment: Naloxone Administration: 2mg intranasal naloxone administered, suspecting opioid involvement. Administered a Nasopharyngeal Airway (NPA) and ventilated with a Bag Valve Mask (BVM).

Patient Response:

The patient regained full consciousness with improved vitals:

GCS: 15

Pulse: 80 bpm

Blood Pressure: 120/80 mmHg

Respiration: 18/min

SPO2: 100%

Patient Admission: Admitted to opioid substance abuse but denied other medical history or allergies.

Transport: Transported for further care after significant improvement.

Scenario 2: Cardiac Arrest

Background

Weather: Cloudy, late-spring evening, 64°F with low humidity.

Dispatch

Reported Incident: An unconscious old male at his home.

Bystander Report: Daughter reported her father collapsed suddenly and became unresponsive

On Scene

Patient: Old adult male in his 70s.

Condition: Unresponsive, no pulse and no respirations

Primary Survey

Airway and Breathing: no respirations

Circulation: no pulse

Mental Status: unconscious.

Other Observations:

Initial Physical Exam

Pulse: **pulseless**

Respiration: **no respirations**

Blood Pressure:

SPO2:

Glucose:

GCS:

Treatment & Transport

Immediate Treatment: Started CPR, attached AED and ventilated with a Bag Valve Mask (BVM).

Shockable rhythm detected and administered a shock.

Patient Response:

The patient had return of spontaneous circulation with improved vitals:

GCS:

Pulse: 50/min

Blood Pressure: 80/50

Respiration: 8/min

SPO2: 84%

Patient Admission: Admitted to previous chest pains

Transport: Transported for further care after significant improvement.

Scenario 3: Opioid

Background

Weather: Dark, late-spring evening, 60°F with low humidity.

Dispatch

Reported Incident: An unconscious female was found in her home.

Bystander Report: Her friend reported that the patient suddenly became unresponsive.

On Scene

Patient: Adult female in her 30s.

Condition: Shallow respirations, unresponsive, pinpoint pupils.

History of Present Incident

Home: Patient and her friend were spending time watching movies at home when the patient became unresponsive.

Primary Survey

Airway and Breathing: Patient is unconscious with shallow respirations.

Circulation: Weak pulse.

Mental Status: Unconscious.

Other Observations: Pill bottles are found nearby, indicating possible opioid overdose.

Initial Physical Exam

Pulse: 60 bpm

Respiration: 6/min

Blood Pressure: 120/72 mmHg

SPO2: 88%

Glucose: 109

GCS: 3

Treatment & Transport

Immediate Treatment: 2mg intranasal naloxone was administered, suspecting opioid involvement.

Also ventilated with a Bag Valve Mask (BVM).

Patient Response:

The patient regained full consciousness with improved vitals:

GCS: 15

Pulse: 75 bpm

Blood Pressure: 120/85 mmHg

Respiration: 12/min

SPO2: 94%

Patient Admission: Admitted to substance abuse but denied other medical history or allergies.

Transport: Transported for further care after significant improvement.

Scenario 4: Respiratory Distress

Background

Weather: Sunny, early-fall morning, 70°F with high humidity.

Dispatch

Reported Incident: A male experiencing breathing difficulties in a hotel lobby.

Bystander Report: The Hotel lobby receptionist mentioned the patient was having a hard time breathing and speaking while distressed.

On Scene

Patient: Adult male in his 70s.

Condition: struggling to breathe. Distressed.

History of Present Incident

3-day history of worsening chest pressure. Difficulty in breathing and abdominal pain. No strenuous activities.

Significant cardiac history with 6 stents, coronary artery bypass and hypertension. Unsure of medications

Primary Survey

Airway and Breathing: Labored breathing

Circulation: irregular radial pulse. EKG indicated ST elevation and sinus tachycardia with frequent premature ventricular contractions suggesting acute cardiac issues

Mental Status: conscious GCS of 15

Other Observations: Skin pale, cool and moist.

Initial Physical Exam

Pulse: 101 bpm

Respiration: 28/min

Blood Pressure: 140/70 mmHg

SPO2: 89%

Glucose: 87

GCS: 15

Pain: 9/10

Treatment & Transport

Treatment: Supplemental oxygen, 324 mg aspirin, 12 lead ecg, and IV were established. Nitroglycerin was administered along with fentanyl administered at 1mcg/kg.

Patient Response:

The patients symptoms improved:

GCS: 15

Pulse: 96 bpm and regular

Blood Pressure: 130/62 mmHg

Respiration: 24/min -

SPO2: 94%

Pain: 4/10

Transport: Transported urgently to a facility capable of cardiac catheterization further care.